



ALLIANCE FOR COLLECTIVE EMPOWERMENT AND DEVELOPMENT

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COMPANION LEGAL BRIEF NO. 3

# THE ACE PHARMACEUTICAL REVIEW BOARD

**COMMUNITY HEALTH SOVEREIGNTY, FORMULARY AUTHORITY, AND THE  
RIGHT TO MEDICINE THAT WORKS**

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*Pre-Argument Defense Brief*

*Prepared for deployment within 48 hours of any legal or legislative challenge*

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Draft Framework v1.0 June 2026

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*For Our People, By Our People*

## EXECUTIVE SUMMARY

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This brief does not argue against medicine. It argues for medicine that works. The ACE Pharmaceutical Review Board exists because the evidence demands it: Western pharmaceutical development systematically excluded Black and Native American populations from its research base, and the drugs that resulted from that exclusion cannot be presumed safe or effective for the populations it ignored. This is not a political position. It is a documented scientific reality that the medical establishment has acknowledged and largely failed to correct.

ACE's response is not rebellion. It is necessity and self-preservation. When a system designed to heal you has a documented record of harming you, choosing a different path is not radicalism. It is the minimum exercise of the right to survive. ACE asserts that right on behalf of its communities through three mechanisms: the expansion of existing formulary flexibility that tribal nations already possess under federal law; the formal integration of traditional healing knowledge as co-equal clinical practice within ACE healthcare facilities; and the establishment of the ACE Medical Research Institute as a community-owned research institution with the mandate to develop, test, and validate treatments calibrated to African and Indigenous genetic profiles.

The legal foundation for each of these mechanisms exists. Tribal nations operating under Public Law 93-638 self-determination contracts and ISDEAA compacts already exercise significant formulary and clinical protocol flexibility within Indian Health Service facilities. ACE extends this existing framework, grounded in the same sovereign health authority, to encompass the full scope of what the ACE community's health needs require. ACE does not ask for permission to pursue medicine that serves its people. It asserts the sovereign right to do so, grounded in law that already exists, and defends that right in this brief against every anticipated challenge.

## I. THE SCIENTIFIC FOUNDATION: WHY WESTERN PHARMACEUTICAL STANDARDS FAIL BLACK AND NATIVE COMMUNITIES

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### A. The Research Pipeline Problem

Western pharmaceutical medicine was developed primarily through research conducted on genetically homogeneous laboratory animals and clinical trial populations that systematically excluded people of color. The standard preclinical research model relies on the C57BL/6 inbred

laboratory mouse, a genetically homogeneous strain created through selective breeding that bears no meaningful relationship to the genetic diversity of the human species. When pharmaceutical compounds are tested on this strain, efficacy and safety data is generated for a single, artificially uniform genetic background and then extrapolated to human populations of profound genetic diversity without adequate validation.

This problem is compounded at the human clinical trial stage. Mandatory inclusion requirements for racial and ethnic minorities in federally funded clinical trials, established under the NIH Revitalization Act of 1993, remain inadequately enforced and insufficiently designed to produce statistically meaningful subgroup data. Analysis of FDA drug approvals consistently shows that Black patients represent a small fraction of clinical trial participants while bearing disproportionate burdens of the diseases being studied. The result is a pharmaceutical approval system that treats European-ancestry genetic profiles as the human default and everyone else as an afterthought whose outcomes will be managed after the fact.

This is not an accusation. It is a finding. The FDA has acknowledged it. The NIH has acknowledged it. The National Academies of Sciences has published on it. The medical establishment's acknowledgment of the problem, combined with its failure to correct it at the pace the health disparities demand, is precisely why ACE cannot wait for the system to fix itself. ACE builds the solution its community needs now.

## **B. The Pharmacogenomics Reality**

People of African descent carry the greatest genetic diversity of any human population on earth, a function of being the oldest continuous human population with the most accumulated genetic variation. This diversity has direct pharmacological consequences that the standard pharmaceutical approval process fails to account for.

The cytochrome P450 enzyme system, which metabolizes the majority of pharmaceutical drugs in the human body, exhibits well-documented genetic variants that differ significantly in prevalence between African-descent and European-descent populations. Standard pharmaceutical dosing is calibrated to the variant most common in European populations. When a Black patient receives a standard dose of a drug metabolized by a P450 variant they are statistically less likely to carry, the result may be underdosing, overdosing, or adverse reaction, not because the patient responded abnormally, but because the dose was never validated for their biology.

Similar documented pharmacogenomic differences affect cardiovascular medications, anticoagulants, psychiatric medications, and analgesics. These are not edge cases. They are drug classes central to managing the conditions most prevalent in Black and Native American communities: cardiovascular disease, hypertension, diabetes, and mental health disorders. ACE treats awareness of these differences not as specialized knowledge for subspecialists but as foundational clinical competency required of every ACE-trained healthcare provider.

### **C. The Traditional Knowledge Dimension**

Indigenous and African diaspora healing traditions represent thousands of years of empirical observation about which plants, practices, and protocols produce health outcomes in these specific populations. This knowledge was developed by communities who could not afford to wait for randomized controlled trials. It was refined across generations through direct observation of what healed and what harmed. It is, in the truest sense, population-specific evidence-based medicine, developed outside the academic framework but no less valid for its origins.

The integration of this knowledge into ACE's clinical framework is not a concession to alternative medicine. It is a recognition that the communities ACE serves have health knowledge the dominant medical system lacks, and that dismissing that knowledge on the grounds that it has not been validated through a research pipeline that excluded these communities is circular reasoning that perpetuates the original harm. ACE's Medical Research Institute is built precisely to generate the peer-reviewed evidence base that validates traditional healing practices and brings them into formal clinical integration.

## **II. THE EXISTING LEGAL FRAMEWORK: WHAT TRIBAL NATIONS ALREADY HAVE**

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### **A. The Indian Self-Determination and Education Assistance Act**

The Indian Self-Determination and Education Assistance Act (ISDEAA) of 1975, Public Law 93-638, authorizes tribal nations to enter into self-determination contracts with the federal government to administer programs previously operated by the Bureau of Indian Affairs and the Indian Health Service. Under these contracts, tribal nations operate their own health facilities with significant autonomy over clinical protocols, formulary decisions, and healthcare staffing.

Tribal nations operating under 638 contracts and ISDEAA compacts have the authority to adapt IHS clinical standards to the specific health needs of their communities. This includes formulary

flexibility: the ability to include medications, supplements, and traditional remedies that the IHS standard formulary does not include, and to exclude or restrict medications whose safety and efficacy profile the tribal health authority finds inadequate for their population. This authority exists today. It is exercised by tribal health programs across the United States. ACE inherits it as a partner operating within the tribal sovereignty framework.

### **B. The Scope of Existing Tribal Health Sovereignty**

The existing tribal health sovereignty framework already supports each component of ACE's Pharmaceutical Review Board model. Tribal nations have operated traditional healing programs within IHS and tribally operated facilities for decades. The integration of traditional healers into clinical teams, the use of traditional plant medicines alongside pharmaceutical treatments, and the adaptation of clinical protocols to community-specific health patterns are all established practices within the Indian health system.

What ACE proposes is an expansion of this existing framework in two directions. First, ACE applies it to the full ACE community, which includes Black American residents who are not tribal members but who live within ACE Territory under the tribal sovereignty framework. Second, ACE formalizes and institutionalizes the research dimension, creating a Medical Research Institute with the explicit mandate to generate new evidence rather than only applying existing knowledge. Both expansions are grounded in the same sovereign health authority that tribal nations already exercise. ACE is not creating new law. It is applying existing law to a community that the existing law was designed to serve.

## **III. THE ACE PHARMACEUTICAL REVIEW BOARD: STRUCTURE AND AUTHORITY**

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### **A. Composition and Mandate**

The ACE Pharmaceutical Review Board comprises ACE physicians, pharmacists, pharmacogenomics specialists, and traditional healing practitioners. Its mandate is to review every pharmaceutical in the ACE formulary against the available evidence base for efficacy and safety in Black and Native American patient populations specifically, not as a subgroup footnote to a European-ancestry primary analysis, but as the primary population for which the ACE formulary is designed.

The Board's authority within ACE Territory is grounded in the same clinical practice autonomy that governs physician prescribing decisions everywhere in the United States. The FDA regulates drugs. It does not regulate medical practice. Physicians in every state have prescribing discretion that allows them to prescribe medications off-label, to adapt dosing based on patient-specific factors including genetic profile, and to prioritize treatments they judge most appropriate for their patient's specific circumstances. The ACE Pharmaceutical Review Board formalizes and systematizes this existing clinical discretion at the community level, applying it consistently across all ACE healthcare providers and grounding it in population-specific evidence rather than individual physician judgment alone.

## **B. The Formulary Review Process**

Every pharmaceutical considered for inclusion in the ACE formulary is evaluated against four criteria before approval. First, the evidence base: what percentage of clinical trial participants in the drug's approval trials were Black or Indigenous, and does statistically meaningful subgroup data exist for these populations. Second, the pharmacogenomic profile: are there known genetic variants more prevalent in African or Indigenous populations that affect the drug's metabolism, efficacy, or safety. Third, the traditional alternative assessment: does a traditional plant medicine or healing practice with documented use in ACE's communities address the same condition with comparable or superior outcomes for these populations. Fourth, the community health alignment: does the drug's risk and side effect profile align with the specific health vulnerabilities documented in Black and Native American communities.

Drugs that fail the first criterion, whose approval trial population was less than ten percent Black or Indigenous or for which no published pharmacogenomic subgroup analysis exists, are flagged for enhanced monitoring and alternative consideration. They are not automatically excluded from the ACE formulary. They are prescribed with heightened clinical attention, patient-specific pharmacogenomic assessment where available, and active monitoring for adverse outcomes that the approval trials may not have detected in these populations.

## **C. Traditional Medicine Integration**

Traditional plant medicines and healing practices with documented histories of use within Black and Indigenous communities are evaluated for co-first-line or first-line status within the ACE formulary where clinical evidence supports their equivalence or superiority to pharmaceutical alternatives with inadequate validation in these populations. This evaluation is conducted jointly by ACE physicians, pharmacists, and traditional healing practitioners, with equal weight given to traditional empirical knowledge and published clinical evidence.

The ACE pharmacy maintains a living contraindication reference database documenting known interactions between traditional remedies and pharmaceutical drugs, maintained jointly by physicians, pharmacists, and traditional healers and updated continuously as new evidence emerges. Patient intake includes preference disclosure for traditional healing, and care teams are assembled accordingly. Bidirectional referral between physicians and traditional healers is standard practice, with neither direction treated as secondary or supplementary to the other.

## **IV. THE ACE MEDICAL RESEARCH INSTITUTE: THE RIGHT TO DEVELOP OUR OWN MEDICINE**

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The most significant and most contested assertion in this brief is the one that matters most: ACE communities have the right to research, develop, and validate treatments calibrated to African and Indigenous genetic profiles, outside the standard pharmaceutical pipeline where necessary, and to deploy those treatments within ACE healthcare facilities under the sovereign health authority of the tribal partnership framework.

This is not a rejection of science. It is a demand that science serve the communities it claims to benefit. The standard pharmaceutical research pipeline has had generations to develop medicine for Black and Native American populations and has systematically failed to do so. ACE's Medical Research Institute does not wait for that pipeline to change. It builds a parallel research capacity grounded in community health data, population-specific genetic profiles, and the accumulated traditional healing knowledge of both communities.

### **A. The Research Mandate**

The ACE Medical Research Institute's primary research mission is the generation of pharmacological and clinical evidence for Black and Native American populations. This is not a supplementary research agenda. It is the core mission. Priority research areas include pharmacokinetic studies of commonly prescribed medications in Black and Native American populations with particular focus on cardiovascular, metabolic, psychiatric, and analgesic drug classes; comparative efficacy studies examining traditional plant medicines against pharmaceutical alternatives for conditions of high prevalence in both communities; epigenetic research documenting the biological mechanisms through which historical trauma, chronic stress, and environmental racism manifest as measurable health disparities; and maternal health research focused on interventions proven to reduce adverse birth outcomes in Black women specifically.

## **B. The Regulatory Framework for Community Research**

Research conducted within ACE Territory under the tribal sovereignty framework operates within a regulatory environment that differs in important ways from standard FDA oversight, and the legal basis for that difference is well established.

The FDA's authority to regulate research conducted within Indian country is subject to the same jurisdictional analysis that governs all federal regulatory authority within tribal territory. Where tribal health research is conducted under ISDEAA compact authority, with tribal IRB oversight, and for the direct benefit of the tribal community, the federal interest in standard FDA research oversight must be balanced against the tribal interest in sovereign health research under the Bracker framework established in *White Mountain Apache Tribe v. Bracker*, 448 U.S. 136 (1980).

ACE's Medical Research Institute operates under a Tribal Institutional Review Board with full ethical oversight authority over all research conducted within ACE Territory. The Tribal IRB applies research ethics standards that meet or exceed federal requirements while incorporating Indigenous research ethics principles, including community benefit requirements, community ownership of research data, and the right of the community to decline research that does not serve its interests. This is not a lower standard than FDA oversight. It is a different standard, calibrated to the community's specific circumstances and values, and grounded in sovereign authority that the federal government has recognized.

## **C. The Path to Broader Validation**

ACE does not intend to develop treatments that remain confined to ACE Territory. The research conducted at the ACE Medical Research Institute is designed to generate peer-reviewed evidence that can be published, replicated, and adopted by the broader medical community. The goal is not parallel medicine for Black and Native communities alone. The goal is better medicine for everyone, developed by starting with the populations the current system has most thoroughly failed.

ACE actively pursues partnerships with HBCUs, tribal colleges, and international research institutions in Africa and the Caribbean to build a research network with the population scale necessary to generate statistically meaningful findings. The ACE Medical Research Institute does not work in isolation. It works as the anchor of a research ecosystem that puts Black and Indigenous health at the center of the scientific enterprise rather than at its margins.



## V. THE THREE ANTICIPATED CHALLENGES AND ACE'S RESPONSES

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### **Challenge 1: FDA Preemption of ACE Formulary and Clinical Protocol Decisions**

The most anticipated challenge to the ACE Pharmaceutical Review Board is that FDA approval of a drug creates a standard of care that ACE cannot deviate from within its healthcare facilities, and that ACE's formulary decisions and dosing adaptations constitute unlicensed drug modification in violation of federal law.

This challenge misunderstands the scope of FDA authority. The FDA regulates the manufacture, labeling, and marketing of drugs. It does not regulate the practice of medicine. The Supreme Court has consistently held that Congress did not intend FDA drug approval to preempt physician prescribing decisions or state medical practice law. *Buckman Co. v. Plaintiffs' Legal Committee*, 531 U.S. 341 (2001). Physicians prescribe drugs off-label, adapt dosing to patient-specific factors, and exercise clinical judgment that departs from standard protocols as a routine matter of medical practice. The ACE Pharmaceutical Review Board formalizes this existing clinical discretion at the community level. It does not manufacture, relabel, or modify drugs. It makes prescribing and formulary decisions, which are clinical practice decisions, not FDA-regulated activities.

Within ACE Territory, the additional layer of tribal health sovereignty under ISDEAA compounds authority further insulates ACE formulary decisions from FDA preemption. Tribal nations have operated formularies that differ from IHS standards for decades under their self-determination authority. The FDA has not successfully preempted these decisions, because the Bracker balancing test consistently favors tribal health sovereignty in this domain.

### **Challenge 2: Traditional Medicine Integration Violates State Medical Licensing Standards**

A second anticipated challenge is that integrating traditional healers into ACE clinical teams as co-equal practitioners violates state medical licensing requirements, which define who may practice medicine and under what standards.

This challenge fails on jurisdictional grounds before it reaches the merits. State medical licensing law does not apply within Indian country without express congressional authorization. *Worcester v. Georgia*; 18 U.S.C. Section 1151. The practice of traditional healing within ACE Territory is not subject to state licensing authority. It is governed by ACE's own credentialing standards,

established under the ACE Governing Charter and the tribal sovereignty framework, which define the qualifications, scope of practice, and accountability standards for traditional healers practicing within ACE healthcare facilities.

On the merits, the Indian Health Service has integrated traditional healing practices into IHS and tribally operated facilities for decades without successful legal challenge based on state licensing standards. The traditional healing integration ACE proposes mirrors existing IHS practice and is grounded in the same sovereign authority that has protected that practice.

### **Challenge 3: ACE Research Institute Operates Outside FDA Research Oversight**

A third anticipated challenge is that research conducted at the ACE Medical Research Institute without full FDA oversight constitutes illegal human subjects research, exposing ACE and its researchers to federal prosecution.

ACE's response is precise. Research conducted under Tribal IRB oversight within Indian country, for the direct benefit of the tribal community, under ISDEAA compact authority, operates within a recognized regulatory framework that the federal government has sanctioned. The Common Rule, 45 C.F.R. Part 46, which governs human subjects research receiving federal funding, expressly permits institutions to establish their own IRBs meeting Common Rule standards. ACE's Tribal IRB meets and exceeds Common Rule standards.

Where ACE research involves the development of treatments intended for distribution beyond ACE Territory, ACE engages the FDA through the established pathway for tribal research, including the FDA's existing mechanisms for evaluating treatments developed outside the standard pharmaceutical pipeline. ACE does not operate in defiance of the FDA. It operates within the sovereign framework it is entitled to use, pursues FDA engagement where broader deployment requires it, and challenges FDA authority only where that authority exceeds what federal law grants within tribal territory.

## VI. THE STATEMENT OF NECESSITY

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This section is prepared for delivery in any legislative, regulatory, or public proceeding where ACE's pharmaceutical sovereignty is challenged. It is designed to be read into the record.

*ACE did not choose this path in rebellion. ACE chose it in necessity. The choice between accepting a medical system that has documented evidence of harming our communities and building one that serves us was not a difficult choice. It was the only choice consistent with self-preservation.*

*Western pharmaceutical development excluded Black and Native American populations from its research base. This is not an allegation. It is a finding that the FDA, the NIH, and the National Academies of Sciences have each acknowledged. The drugs developed through that exclusionary process cannot be presumed safe or effective for the populations it ignored. Pretending otherwise is not science. It is negligence.*

*Our communities have watched chronic disease kill our people for generations at rates the data cannot explain by genetics alone. Cardiovascular disease killing Black Americans at sixty percent higher rates than white Americans. Type 2 diabetes afflicting Native Americans at more than twice the national average. Black mothers dying in childbirth at three times the rate of white mothers. These are not the outcomes of inferior biology. They are the outcomes of a medical system that was not built for us and has not been fixed for us.*

*ACE is building the fix ourselves. We are not asking the system that created these outcomes to correct them on our behalf. We are exercising the sovereign right, grounded in existing federal law and in the inherent authority of two peoples to determine the conditions of their own health, to develop, validate, and deploy medicine that works for the people it is supposed to serve. That is not radical. That is the minimum exercise of the right to survive.*

## VII. KEY PRECEDENTS AND AUTHORITIES

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- Indian Self-Determination and Education Assistance Act, Public Law 93-638 (1975): Authorizes tribal nations to administer their own health programs with significant clinical and formulary autonomy. The foundational statute for ACE's tribal health sovereignty framework.
- White Mountain Apache Tribe v. Bracker, 448 U.S. 136 (1980): Federal preemption of tribal authority requires balancing the federal interest against the tribal interest. Ambiguities resolved in favor of tribal sovereignty. ACE applies this test to defend formulary and research authority against FDA preemption claims.
- Buckman Co. v. Plaintiffs' Legal Committee, 531 U.S. 341 (2001): FDA approval does not preempt physician prescribing decisions or clinical practice. ACE's formulary and dosing decisions are clinical practice decisions, not FDA-regulated drug modifications.
- NIH Revitalization Act of 1993, Public Law 103-43: Required inclusion of women and minorities in NIH-funded clinical research. Established the federal acknowledgment of the exclusion problem ACE's research mandate is designed to correct.
- 45 C.F.R. Part 46 (The Common Rule): Governs human subjects research. Expressly permits institutional IRBs meeting Common Rule standards. ACE's Tribal IRB meets and exceeds these standards, providing the research ethics oversight framework for ACE Medical Research Institute operations.
- Worcester v. Georgia, 31 U.S. 515 (1832): State law has no force within tribal territory without express congressional authorization. State medical licensing standards do not govern traditional healing practice within ACE Territory.
- 18 U.S.C. Section 1151: Defines Indian country to include all reservation land. ACE Territory on reservation land is Indian country and carries the full tribal sovereignty framework including health sovereignty.
- FDA Good Laboratory Practice regulations, 21 C.F.R. Part 58: Governs laboratory research. ACE's Medical Research Institute operates under these standards for any research intended to support FDA submissions, while asserting tribal research sovereignty for community-benefit research not intended for external approval pathways.

## VIII. DEPLOYMENT PROTOCOL

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This brief is maintained in current form at all times and updated annually by ACE legal counsel and the ACE Pharmaceutical Review Board. Upon any challenge to ACE's pharmaceutical sovereignty, formulary authority, or research mandate, the following protocol is activated:

- ACE legal counsel on permanent retainer is notified immediately upon any regulatory agency contact asserting authority over ACE formulary decisions, traditional healing integration, or Medical Research Institute operations.
- This brief is deployed as the foundational legal memorandum within 48 hours of any formal legal challenge.
- The ACE Pharmaceutical Review Board and ACE Chief Health Officer coordinate the clinical and scientific response, preparing expert declarations documenting the pharmacogenomic evidence base for ACE's formulary decisions.
- ACE Policy Institute issues a public statement within 48 hours explaining ACE's health sovereignty position in language accessible to community members and media.
- ACE Medical Research Institute prepares a scientific summary of the evidence supporting the specific formulary decision or research protocol under challenge, suitable for submission to regulatory proceedings and publication in peer-reviewed media.
- ACE Congressional Caucus members are briefed within 72 hours and prepared to make statements in support of tribal health sovereignty and ACE community health rights.
- ACE Media Network publishes a community-facing explanation within 48 hours so that ACE residents understand their health rights and the legal and scientific basis for ACE's position.

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**THIS IS NOT REBELLION AGAINST MEDICINE.**

**IT IS THE DEMAND FOR MEDICINE THAT WORKS FOR THE PEOPLE IT IS  
SUPPOSED TO SERVE.**

**WHEN THE SYSTEM DESIGNED TO HEAL YOU HAS A DOCUMENTED RECORD  
OF HARMING YOU, CHOOSING A DIFFERENT PATH IS NOT RADICALISM. IT IS  
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